

August 12, 2026

BlueChoice HealthPlan of South Carolina
Provider Appeals Unit
P.O. Box 6170
Columbia, SC 29260

RE: Formal Appeal of Claim Denial — Raeann O'Kon (MRN MUSC4885131), Claim MUSC-20251229-881F-1

Patient	Raeann O'Kon (DOB 1968-10-09)
MRN	MUSC4885131
Member ID / Group	BLU809171647 / GRP-65822
Claim number	MUSC-20251229-881F-1
Date of service	2025-12-29
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$465.72
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-04-23

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health submits this first-level reconsideration on behalf of Raeann O'Kon (Member ID: BLU809171647, Group: GRP-65822) regarding the denial of Claim MUSC-20251229-881F-1, date of service December 29, 2025. BlueChoice HealthPlan of South Carolina denied the claim under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") with RARC N115, stating that "documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." The total denied amount is \$465.72 for one unit of CPT 99214, an established outpatient office visit of moderate complexity rendered by Sarah E. Whitmore, MD (NPI 1447382910) at MUSC Health Ashley River Tower. MUSC Health respectfully disagrees with this determination and requests full reversal and payment at the contracted allowed amount of \$290.99.

CLINICAL BACKGROUND

Ms. O'Kon is a 57-year-old established patient presenting on December 29, 2025, at MUSC Health's on-campus outpatient facility (Place of Service 22) with a primary diagnosis of acute upper respiratory infection, unspecified (J06.9). Her active medication profile is clinically significant: she carries a longstanding prescription for Metformin hydrochloride 500 MG Extended Release, active since 2006, consistent with management of type 2 diabetes mellitus (secondary diagnosis E11.9 reported on this claim), and Hydrochlorothiazide 25 MG, active since 2003, consistent with hypertension management. The co-existence of an acute upper respiratory infection in a patient with type 2 diabetes and a chronic diuretic regimen represents a clinical scenario requiring careful evaluation. Intercurrent infections in diabetic patients carry elevated risk for glycemic destabilization and secondary complications, and the presence of an active diuretic further warrants assessment of hydration status and electrolyte balance during an acute illness. These factors, taken together, support the medical necessity of a moderate-complexity evaluation and management visit.

BASIS FOR APPEAL

CPT 99214 is appropriately assigned to an established patient office visit requiring moderate medical decision-making or at least 30 minutes of total time. The presenting diagnosis of acute upper respiratory infection in the context of active, chronic conditions managed with ongoing prescription medications — specifically type 2 diabetes and a condition requiring long-term diuretic therapy — satisfies the criteria for moderate complexity under current AMA CPT evaluation and management guidelines. The number and complexity of problems addressed, the review and management of active prescription medications, and the potential for complications arising from an acute illness superimposed on chronic metabolic disease collectively support the level of service billed.

The payer's denial references a Local Coverage Determination as the basis for the medical necessity finding. However, LCDs are instruments of Medicare policy administered by Medicare Administrative Contractors and do not govern commercial HMO claims adjudication under a BlueChoice HealthPlan of South Carolina Open Access plan. Applying an LCD standard to a commercial claim is not a recognized basis for denial under the terms of a commercial insurance contract, and MUSC Health requests that the claim be re-adjudicated under the plan's applicable commercial medical necessity criteria. The encounter documentation, including the reported diagnoses and the patient's active chronic medication regimen, substantiates that the visit was both reasonable and necessary.

REQUESTED ACTION

MUSC Health respectfully requests that BlueChoice HealthPlan of South Carolina reverse the medical necessity denial for Claim MUSC-20251229-881F-1, CPT 99214, date of service December 29, 2025, and issue payment at the contracted allowed amount of \$290.99. Should the plan require additional clinical documentation from the rendering provider, Dr. Sarah E. Whitmore, MD, MUSC Health will promptly coordinate its submission. If this reconsideration is not resolved in favor of the provider, MUSC Health reserves the right to pursue a second-level written appeal to the Provider Appeals Unit within the timeframes established by the plan.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record